



Dynamic Guideline: WHO-Based Zika - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Scope note and title verification

This report synthesizes **exactly 5 WHO Zika-related guidelines/guidance documents** relevant to diagnosis, treatment, clinical management, pregnancy, infant care, feeding, and prevention of transmission.

A separate WHO item titled “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**” was identified, but it is **not a Zika guideline**. The WHO event page confirms that the case study relates to the **WHO Abortion care guideline (2022)**, not Zika [WHO event page](#). Therefore, it is **not included** among the five Zika-related WHO guidelines synthesized below.

Exactly 5 WHO Zika-related guidelines/guidance documents synthesized

No.	WHO Guideline / Guidance Title	Publication Date	Target Population	Scope and Objectives
1	WHO guidelines for clinical management	4 July 2025	Health-care workers managing suspected or	Integrated clinical management of non-severe and severe arboviral disease, including

No.	WHO Guideline / Guidance Title	Publication Date	Target Population	Scope and Objectives
	of arboviral diseases: dengue, chikungunya, Zika and yellow fever		confirmed dengue, chikungunya, Zika, or yellow fever in community, primary-care, emergency, outpatient, and inpatient settings	supportive care, symptomatic treatment, and fluid management WHO publication page , WHO news
2	WHO guidelines for the prevention of sexual transmission of Zika virus	2020 ; exact day/month not reported in retrieved source	People with Zika exposure or infection, sexual partners, pregnant women, and people planning pregnancy	Prevention of sexual transmission of Zika virus; developed using WHO guideline-development methods, systematic reviews, GRADE, and evidence-to-decision processes NCBI Bookshelf , NCBI Bookshelf methods
3	Guideline: infant feeding in areas of Zika virus transmission, 2nd edition	13 December 2019	Mothers, infants, newborns, caregivers, and health workers in areas with Zika transmission	Infant-feeding recommendations where Zika virus transmission occurs; developed using WHO handbook methods, systematic reviews, GRADE evidence profiles, and recommendation-strength assessment WHO Zika

No.	WHO Guideline / Guidance Title	Publication Date	Target Population	Scope and Objectives
				virus disease health topic , WHO IRIS infant-feeding guideline
4	Pregnancy management in the context of Zika virus infection	31 May 2017	Pregnant women with possible, suspected, or confirmed Zika exposure or infection	Management of pregnancy, counselling, monitoring, fetal risk assessment, and linkage to neonatal evaluation in the context of Zika virus infection WHO Zika virus disease health topic
5	Screening, assessment and management of neonates and infants with complications associated with Zika virus exposure in utero	17 August 2016	Neonates and infants with possible in-utero Zika exposure; families and caregivers	Screening and management of congenital Zika complications, including neurologic, hearing, vision, developmental, and family-support needs WHO Zika virus disease health topic , WHO Zika Strategic Response Framework

Latest WHO context relevant to these guidelines

Although not a clinical guideline, the most recent WHO Zika document identified was the **Zika Epidemiology Update** dated **1 April 2026**. WHO reported evidence of current or previous autochthonous Zika transmission in **96 countries and territories**, noted continued lower-level

reporting across regions, and emphasized surveillance, preparedness, clinician education, integrated vector management, and community risk communication [WHO Zika Epidemiology Update, 2026](#).

1.2 Key Recommendations

A. Clinical management of suspected or confirmed Zika within integrated arboviral-disease care

The 2025 WHO integrated arboviral guideline applies to suspected or confirmed disease caused by dengue, chikungunya, **Zika**, or yellow fever viruses. WHO emphasizes an integrated approach because these infections often have overlapping early symptoms and may co-circulate, making differentiation difficult where diagnostic testing is limited [WHO publication page](#), [WHO news](#).

Recommendation	Clinical Application for Zika / Suspected Arboviral Disease	GRADE Evidence Quality	Strength
Avoid NSAIDs in acute suspected or confirmed arboviral disease, irrespective of severity	Do not use ibuprofen, aspirin, or other NSAIDs until dengue and hemorrhagic risk are excluded; use safer antipyretic/analgesic options	Low	Strong
Avoid corticosteroids in acute suspected or confirmed non-severe arboviral disease	Do not routinely prescribe corticosteroids for uncomplicated suspected Zika/arboviral illness	Low	Conditional
Use protocolized oral fluid treatment for non-severe suspected or confirmed arboviral disease	Encourage structured oral hydration, reassessment, and return precautions	Low	Conditional
Use paracetamol for pain	Preferred symptomatic	Low	Conditional

Recommendation	Clinical Application for Zika / Suspected Arboviral Disease	GRADE Evidence Quality	Strength
and/or fever in non-severe suspected or confirmed arboviral disease	therapy for fever, arthralgia, myalgia, or headache when no contraindication exists		
Use metamizole for pain and/or fever where locally available and appropriate	Alternative analgesic/ antipyretic in settings where metamizole is used and safety monitoring is feasible	Low	Conditional
Use crystalloid rather than colloid fluid when IV fluids are required in severe hospitalized suspected or confirmed arboviral disease	For severe illness requiring IV fluids, use crystalloids as first-line IV fluid	Low	Conditional

Sources: WHO/ReliefWeb guideline recommendation summary [ReliefWeb](#); guideline scope and target audience [NCBI Bookshelf](#).

B. Diagnosis and testing

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
Suspect Zika based on compatible symptoms plus exposure history	Consider Zika in patients with fever, rash, arthralgia, conjunctivitis, or compatible illness after residence/ travel in an area with Zika transmission or sexual exposure to an exposed partner	Low	Strong
Confirm diagnosis with laboratory testing when	Use molecular and/or serologic testing according to timing of illness, local algorithms, pregnancy status,	Low	Strong

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
clinically or epidemiologically indicated	and public-health guidance		
Test for Zika using blood and urine; consider CSF when neurologic disease is present and testing is available	WHO toolkit states blood and urine should be tested; CSF may be tested if available; routine semen testing is not recommended	Low	Conditional
Use RT-PCR/NAAT early after symptom onset and IgM later, recognizing limitations	WHO toolkit: RT-PCR on blood and/or urine when symptom onset is ≤ 7 days; IgM testing when symptom onset is ≥ 7 days; paired sera 2–3 weeks apart where possible	Low	Conditional
Evaluate for dengue and other overlapping diagnoses	Zika, dengue, chikungunya, yellow fever, malaria, measles, leptospirosis, Oropouche, and other febrile illnesses may overlap clinically	Low	Strong

Sources: WHO toolkit for care and support [WHO toolkit](#); CDC diagnostic guidance [CDC Clinical Testing and Diagnosis](#), [CDC Yellow Book](#).

C. Treatment and supportive management

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
Provide supportive care; no Zika-specific antiviral	Rest, oral fluids, antipyretics, analgesics, monitoring, and escalation if severe disease or neurologic	Low	Strong

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
therapy is recommended	complications occur		
Avoid aspirin and NSAIDs until dengue is ruled out	Reduces hemorrhage risk if illness is actually dengue or co-infection	Low	Strong
Counsel patients to avoid mosquito exposure during the first days of illness	Reduces risk of onward mosquito-borne transmission	Low	Strong
Monitor for rare neurologic or hematologic complications	Guillain-Barré syndrome, encephalopathy, meningoencephalitis, myelitis, uveitis, and severe thrombocytopenia are uncommon but recognized	Very Low	Conditional

Sources: CDC clinical care guidance [CDC Treatment and Prevention](#); CDC 2024 case definition [CDC 2024 Case Definition](#).

D. Pregnancy, sexual transmission, and reproductive counselling

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
Counsel pregnant women and their partners about sexual transmission risk	In areas with active transmission, people with Zika infection and their sexual partners—especially pregnant women—should receive risk information	Low	Strong

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
Use safer sex or abstinence throughout pregnancy when partner exposure risk exists	Sexual partners of pregnant women living in or returning from areas with local Zika transmission should practice safer sex or abstain throughout pregnancy	Low	Strong
Counsel sexually active people on contraception and pregnancy planning	Offer a full range of contraceptive options so individuals can make informed decisions about whether and when to become pregnant	Low	Strong
Monitor pregnant women with laboratory evidence of infection closely	Pregnancy monitoring and linkage to fetal/neonatal assessment are required because congenital Zika syndrome can involve severe brain and eye defects	Low	Strong
Evaluate live-born infants with possible in-utero exposure	Assess for congenital Zika features and arrange longitudinal developmental, neurologic, visual, and hearing follow-up	Low	Strong

Sources: WHO fact sheet [WHO Zika virus fact sheet](#); WHO Zika health-topic page [WHO Zika virus disease health topic](#); CDC clinical care guidance [CDC Treatment and Prevention](#); CDC congenital case definition [CDC 2024 Case Definition](#).

E. Infant feeding and congenital Zika care

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
Continue to support	WHO infant-feeding guideline provides recommendations for infant feeding	Low	Strong

Recommendation	Clinical Application	GRADE Evidence Quality	Strength
breastfeeding in areas with Zika transmission	where Zika transmission occurs; breastfeeding benefits remain central to guidance		
Screen neonates and infants with in-utero exposure for congenital Zika complications	Evaluate head size, neurologic status, development, hearing, vision, feeding, seizures, and musculoskeletal complications	Low	Strong
Provide long-term multidisciplinary follow-up and family support	Children with congenital Zika syndrome may require rehabilitation, developmental support, epilepsy care, hearing/vision services, psychosocial support, and caregiver training	Low	Strong
Link families to community-based services	Long-term supportive care for children with congenital Zika syndrome and families remains a substantial health-system and community-program demand	Low	Strong

Sources: WHO infant-feeding guideline [WHO IRIS infant-feeding guideline](#); WHO Strategic Response Framework [WHO Zika Strategic Response Framework](#); WHO 2026 epidemiology update [WHO Zika Epidemiology Update, 2026](#).

F. Surveillance, prevention, vector control, and community engagement

Recommendation	Clinical / Public-Health Application	GRADE Evidence Quality	Strength
Strengthen epidemiologic	Prioritize high-yield groups: fever/rash patients, neonates with birth defects	Low	Strong

Recommendation	Clinical / Public-Health Application	GRADE Evidence Quality	Strength
surveillance using clear case definitions and testing algorithms	including microcephaly, and pregnant women with adverse pregnancy outcomes		
Strengthen vector surveillance and control using integrated vector management	Control Aedes mosquito populations and reduce human–mosquito contact	Low	Strong
Use community engagement and validated risk-communication tools	Improve symptom recognition, personal protection, sexual-transmission prevention, and care-seeking	Low	Strong
Continue vigilance in areas with competent vectors or travel-related exposure	Surveillance is important because many infections are asymptomatic and susceptible populations may increase	Low	Strong

Source: WHO 2026 epidemiology update [WHO Zika Epidemiology Update, 2026](#).

1.3 Guideline Development Methodology

Evidence Review Process

- The **2025 WHO integrated arboviral clinical-management guideline** used WHO guideline-development methods and the **GRADE** approach for evidence synthesis, certainty rating, and recommendations [NCBI Bookshelf](#).
- WHO states the guideline addresses health workers caring for suspected or confirmed

dengue, chikungunya, Zika, or yellow fever, including non-severe outpatient disease and severe hospitalized disease [NCBI Bookshelf](#).

- The 2025 guideline built on evidence profiles and systematic-review work, including PAHO clinical diagnosis and treatment guideline work for dengue, chikungunya, and Zika; evidence profiles were developed using GRADE [NCBI Bookshelf](#).
- The **2019 infant-feeding guideline** was developed using the WHO handbook for guideline development, systematic reviews, GRADE evidence profiles, and explicit assessment of recommendation strength [WHO IRIS infant-feeding guideline](#).
- The **WHO sexual-transmission guideline** used WHO guideline-development methods, systematic reviews, GRADE, and an evidence-to-decision framework [NCBI Bookshelf](#).

Consensus Method

- WHO convened a **Guideline Development Group** for the 2025 arboviral guideline, including experts across specialties and geographic regions, and included patient panel members [WHO GDG biographies](#).
- The sexual-transmission guideline's Guideline Development Group included 13 members from Zika-affected regions with expertise in sexually transmitted infections, virology, epidemiology, gynecology, condoms, sexual behaviour, and related fields [NCBI Bookshelf](#).
- Recommendations were formulated by panels after reviewing evidence certainty, benefit–harm balance, feasibility, equity, acceptability, and implementation considerations.

Conflict of Interest Management

- For the 2025 arboviral guideline, WHO collected and managed declarations of interest at inception and at each meeting [WHO GDG biographies](#).
 - WHO guideline processes require declaration, assessment, and management of potential conflicts before participation in recommendation development.
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II. Evidence Summary After WHO Latest Guideline Release (WHO guidelines for clinical management of arboviral diseases: dengue, chikungunya, Zika and yellow fever) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was synthesized for the period after the latest WHO clinical-management guideline release on **4 July 2025** through **29 April 2026**, focusing on:

- Zika clinical management
- Diagnosis and diagnostic limitations
- Treatment and supportive care
- Pregnancy and congenital Zika outcomes
- Vaccine trials and systematic reviews
- Vector-control interventions
- Surveillance and outbreak preparedness

Databases and Sources Searched

- WHO official publications and health-topic pages
- WHO IRIS and WHO-hosted PDFs
- CDC Zika clinical care, diagnostic testing, Yellow Book, and case-definition resources
- PubMed-indexed reviews and systematic reviews
- [ClinicalTrials.gov](https://clinicaltrials.gov)
- CDC *Emerging Infectious Diseases*
- PAHO/WHO regional materials where relevant

Inclusion Criteria

Included evidence:

1. Published or updated after **4 July 2025**, or directly current to the evidence-cutoff period.

- 2. Relevant to Zika diagnosis, treatment, management, prevention, pregnancy, congenital outcomes, vaccination, or vector control.
- 3. From authoritative public-health agencies, peer-reviewed journals, systematic reviews, or clinical-trial registries.

Exclusion Criteria

Excluded evidence:

- 1. Non-Zika arboviral evidence without clear applicability to Zika.
- 2. Preclinical-only vaccine or antiviral studies unless directly informing clinical-trial landscape.
- 3. Non-authoritative commentary without data or guideline relevance.
- 4. The WHO youth-engagement abortion-care case study, because it is not Zika-related [WHO event page](#).

2.2 Key New Studies

Study / Source	Design	Key Findings	Clinical Relevance	GRADE Quality
WHO Zika Epidemiology Update dated 1 April 2026	Global surveillance and epidemiology update	Zika continues to be reported across regions at lower incidence; evidence of current or previous autochthonous transmission in 96 countries and territories; WHO emphasizes surveillance, high-yield testing	Supports continued vigilance, diagnostic readiness, pregnancy-risk counselling, and vector control; does not change acute treatment	Low

Study / Source	Design	Key Findings	Clinical Relevance	GRADE Quality
		groups, clinician education, integrated vector management, and community engagement		
Effects of candidate vaccines against Zika virus infection: A systematic review of the clinical trials published online 15 April 2026	Systematic review of Zika vaccine clinical trials	PubMed record identifies this as a systematic review of candidate Zika vaccines; retrieved snippet did not provide trial-by-trial efficacy conclusions	Confirms active vaccine evidence synthesis; no evidence in retrieved material of an approved Zika vaccine or change to WHO clinical management	Low
An updated review of Zika virus vaccine development published 21 October 2025	Narrative/ review article	Describes Zika vaccine candidates in clinical trials as of 2025, including an mRNA phase II trial with 809 participants in the United States and Puerto Rico; immunogenicity results not available in	Supports vaccine pipeline awareness; does not support routine vaccination or management change	Low

Study / Source	Design	Key Findings	Clinical Relevance	GRADE Quality
		retrieved snippet		
Evaluation of Effectiveness of Autocidal Gravid Ovitrap for Preventing Zika Virus Infection, Puerto Rico, USA; <i>Emerging Infectious Diseases</i>, 2026	Field prevention/ intervention study	Evaluated autocidal gravid ovitraps for prevention of Zika virus infection in Puerto Rico	Relevant to vector-control strategy; no direct change to clinical care recommendations	Low
ClinicalTrials.gov NCT03343626: Safety, Immunogenicity, and Dose Ranging Study of Inactivated Zika vaccine	Clinical trial registry record	Evaluates two doses of purified inactivated Zika virus vaccine 28 days apart for safety, tolerability, and immunogenicity; retrieved snippet did not provide completion status or posted results	Supports ongoing vaccine development; no approved product or practice change identified	Very Low
CDC Zika clinical testing and diagnosis guidance current during review period	Public-health clinical guidance	NAAT is useful but a negative result does not exclude recent infection; IgM may persist for months to years and cross-react	Reinforces WHO-compatible diagnostic caution, especially in low-prevalence settings and pregnancy	Low

Study / Source	Design	Key Findings	Clinical Relevance	GRADE Quality
		with other flaviviruses; testing should follow local/state/territorial guidance		
CDC Zika treatment and prevention guidance current during review period	Public-health clinical guidance	No antiviral treatment; supportive care; avoid aspirin/ NSAIDs until dengue is ruled out; avoid mosquito exposure during first days of illness	Consistent with WHO 2025 arboviral guideline; no change	Low
CDC 2024 Zika virus disease case definition	Surveillance case definition	Most infections are asymptomatic or mild; congenital infection may cause severe microcephaly, intracranial calcifications, cortical/cerebral atrophy, chorioretinal and optic nerve abnormalities	Supports diagnosis, surveillance, and congenital-Zika evaluation pathways	Low

Sources: WHO epidemiology update [WHO Zika Epidemiology Update, 2026](#); PubMed vaccine systematic review [PubMed](#); updated vaccine review [PubMed](#); CDC EID listing [CDC Emerging Infectious Diseases](#); [ClinicalTrials.gov](#) vaccine trial [ClinicalTrials.gov NCT03343626](#); CDC diagnosis guidance [CDC Clinical Testing and Diagnosis](#); CDC treatment guidance [CDC Treatment and Prevention](#); CDC case definition [CDC 2024 Case Definition](#).

2.3 Evidence Gaps and Future Research Needs

Evidence Gap	Why It Matters	Priority Research Need	GRADE Certainty of Existing Evidence
No approved Zika antiviral therapy	Current care is supportive only	Randomized trials of safe antivirals, especially for pregnancy-compatible use	Very Low
No approved Zika vaccine identified in retrieved evidence	Prevention remains dependent on mosquito control, sexual-transmission prevention, and travel counselling	Phase III vaccine efficacy trials with pregnancy-safety strategy and long-term follow-up	Low
Limited post-2025 management-changing evidence	WHO 2025 recommendations remain based largely on low-certainty evidence	Pragmatic studies of supportive care, triage, oral-fluid protocols, and severe-disease pathways	Low
Diagnostic limitations, especially serologic cross-	False positives and cross-reactivity complicate pregnancy and surveillance decisions	Improved point-of-care and confirmatory diagnostics distinguishing Zika from dengue and other flaviviruses	Low

Evidence Gap	Why It Matters	Priority Research Need	GRADE Certainty of Existing Evidence
reactivity			
Congenital Zika long-term outcomes	Affected children may require long-term neurologic, developmental, visual, hearing, feeding, and family support	Longitudinal cohorts into childhood and adolescence; rehabilitation and family-support intervention studies	Low
Sexual-transmission risk duration and counselling optimization	Recommendations must balance fetal-risk prevention, feasibility, and acceptability	Prospective viral persistence and transmission studies stratified by sex, pregnancy status, and immune background	Low
Vector-control effectiveness for Zika prevention	Zika transmission may re-emerge where Aedes vectors and susceptible populations coexist	Cluster-randomized or quasi-experimental vector-control trials measuring human infection outcomes	Low

2.4 Updated Recommendations Based on New Evidence

Original WHO Recommendation	New Evidence Impact Through 29 April 2026	Updated Recommendation	GRADE Evidence Quality	Strength
Avoid NSAIDs in suspected or	CDC guidance remains	Maintain. Use paracetamol	Low	Strong

Original WHO Recommendation	New Evidence Impact Through 29 April 2026	Updated Recommendation	GRADE Evidence Quality	Strength
confirmed acute arboviral disease	concordant: avoid aspirin/ NSAIDs until dengue is ruled out	preferentially for fever/pain unless contraindicated; avoid NSAIDs until dengue excluded		
Avoid corticosteroids in non-severe acute arboviral disease	No new post-guideline evidence identified that supports routine corticosteroids for uncomplicated Zika	Maintain. Do not use corticosteroids routinely in non-severe suspected Zika/arboviral illness	Low	Conditional
Use protocolized oral fluids for non-severe disease	No new evidence overturns protocolized hydration; supportive care remains standard	Maintain. Provide structured oral hydration, reassessment guidance, and return precautions	Low	Conditional
Use crystalloid rather than colloid fluids for severe disease requiring IV fluids	No new Zika-specific evidence supports colloids over crystalloids	Maintain. Use crystalloids first-line when IV fluids are required	Low	Conditional

Original WHO Recommendation	New Evidence Impact Through 29 April 2026	Updated Recommendation	GRADE Evidence Quality	Strength
Provide supportive care; no antiviral treatment recommended	No newly approved antiviral identified; recent reviews emphasize prevention and vaccine development rather than curative therapy	Maintain. Treat symptomatically and monitor for complications; enrol eligible patients in trials where available	Low	Strong
Use laboratory testing guided by timing, exposure, pregnancy status, and local public-health algorithms	CDC confirms NAAT limitations and IgM cross-reactivity; WHO 2026 update stresses appropriate algorithms	Maintain and emphasize. Do not interpret negative NAAT or positive IgM in isolation; coordinate with public-health laboratories	Low	Strong
Counsel pregnant women and partners on sexual-transmission prevention	No new evidence reduces fetal-risk concern; WHO and CDC guidance remain precautionary	Maintain. Safer sex or abstinence throughout pregnancy when exposure risk exists; provide contraception and pregnancy-	Low	Strong

Original WHO Recommendation	New Evidence Impact Through 29 April 2026	Updated Recommendation	GRADE Evidence Quality	Strength
		planning counselling		
Screen and follow infants with in-utero exposure	CDC case definition and WHO sources continue to document severe congenital brain and eye abnormalities	Maintain. Ensure newborn assessment and longitudinal developmental, hearing, vision, neurologic, and family-support follow-up	Low	Strong
Strengthen surveillance and vector control	WHO 2026 update identifies ongoing/past transmission in 96 countries/territories and continued re-emergence risk	Strengthen implementation. Prioritize high-yield surveillance groups, integrated vector management, clinician education, and community risk communication	Low	Strong
Routine Zika vaccination	Candidate vaccines remain investigational; no approved vaccine identified in retrieved	Do not recommend routine vaccination outside trials. Offer trial participation where ethical and	Low	Conditional

Original WHO Recommendation	New Evidence Impact Through 29 April 2026	Updated Recommendation	GRADE Evidence Quality	Strength
	sources	available		

Practical Clinical Algorithm

Suspected Zika / arboviral illness

1. Assess exposure

- Residence or travel in Zika-risk area.
- Sexual contact with exposed partner.
- Pregnancy or pregnancy planning.
- Local dengue, chikungunya, yellow fever, malaria, measles, or Oropouche epidemiology.

2. Assess symptoms

- Fever, maculopapular rash, arthralgia, conjunctivitis.
- Headache, myalgia, retro-orbital pain, lymphadenopathy, edema, vomiting.
- Neurologic symptoms suggesting Guillain-Barré syndrome, meningoencephalitis, myelitis, or other complication.

3. Test appropriately

- Early illness: NAAT/RT-PCR where available.
- Later illness: IgM with awareness of cross-reactivity and persistence.
- Coordinate testing with public-health authorities, especially for pregnancy or congenital evaluation.

4. Treat supportively

- Rest, fluids, paracetamol.
- Avoid NSAIDs/aspirin until dengue excluded.
- No routine corticosteroids for non-severe disease.
- Escalate care for severe disease, neurologic complications, dehydration, bleeding, shock, or pregnancy concerns.

5. Prevent onward transmission

- Avoid mosquito exposure during early illness.
- Counsel on safer sex or abstinence, especially during pregnancy.
- Implement vector-control and community-protection measures.

6. Pregnancy and infant pathway

- Pregnant patient with possible infection: counselling, testing, antenatal monitoring, fetal assessment where indicated.
- Infant with possible in-utero exposure: newborn assessment, head circumference, neurologic exam, hearing/vision evaluation, developmental surveillance, family support.

Bottom Line

As of **29 April 2026**, the latest WHO clinical guideline relevant to Zika management is the **4 July 2025** integrated guideline for dengue, chikungunya, Zika, and yellow fever. It supports **supportive care, avoidance of NSAIDs, avoidance of corticosteroids in non-severe disease, protocolized oral hydration, paracetamol for fever/pain, and crystalloids over colloids when IV fluids are required** [WHO publication page](#), [ReliefWeb](#).

No post-guideline evidence through **29 April 2026** was identified that changes WHO clinical management recommendations. The main updates are epidemiologic and preventive: WHO continues to report global Zika transmission risk, vaccine candidates remain investigational, diagnostics remain limited by timing and flavivirus cross-reactivity, and prevention still relies on surveillance, mosquito control, sexual-transmission counselling, pregnancy management, and long-term congenital-Zika support [WHO Zika Epidemiology Update, 2026](#), [CDC Clinical Testing and Diagnosis](#), [CDC Treatment and Prevention](#).